



Clinical Documentation

Allied Health Practice Guideline #22

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Allied Health	Dietetics, Occupational Therapy, Physiotherapy, Podiatry, Psychology, Social Work and Speech Pathology

Introduction

Clinical documentation provides a legal record of the Allied Health interventions that have been provided to patients or their family/carer, including assessment, treatment, consultation, education and discharge planning.

Risk Statement

Non-compliance with this Guideline will:

Breach legislative requirements	☒	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☒	Impact on Patient Safety	☐
Breach professional standards	☒	Misconduct	☐
Breach SCGOPHCG Mission & Values	☒	Staff Safety	☐
Other:	☐		

Guideline Details

Requirements for paper-based medical records and forms

All paper forms must include a patient addressograph label containing the DMR 2D barcode. The DMR 2D barcode identifies the episode of care folder that the form will be filed in. Labels from the patient's historic episodes of care MUST NOT be used.

Allied Health clinical documentation will:

- include a 3-point ID check notation
- be clear, accurate, objective and legible
- use only black/blue pen
- include date of entry (using dd/mm/yy) in the margin
- include time of entry (using 24-hour clock – hh:mm) in the margin: entries should be made at the time of an intervention or as soon as possible afterwards – late entries should be identified as a “late entry” or “written in retrospect” and document the time (and date) of the completed intervention within the entry
- include a designation sticker in the margin/use profession specific paper
- record a summary of interventions according to professions specific guidelines
- not leave blank lines or partially blank lines – line through empty space
- use medical record approved stickers
- notate all communication with the family/carer regarding care planning

- minimise use of abbreviations, acronyms, contractions and symbols where possible; if used, they must be defined the first time in each entry eg Total Hip Replacement (THR)
- be signed by the author, and include their printed name and contact details
 - For inpatients, page/extension contact details should be used on all entries
 - For outpatients, page/extension details should be used on at least initial and discharge summaries

If an entry has been written in error, the original incorrect entry must remain readable. Do not overwrite the incorrect entries or use correction fluid/tape. Draw a line through the incorrect entry; document “written in error” followed by the author’s printed name.

Requirements for digital medical records

Allied Health clinical documentation will:

- use discipline specific progress notes
- include a 3-point ID check notation
- be clear, accurate and objective
- include an automatic record of date and time of entry – late entries should be identified as a “late entry” or “written in retrospect” and document the time (and date) of the completed intervention within the entry
- include an automatic designation
- record a summary of interventions according to professions specific guidelines
- notate all communication with the family/carer regarding care planning
- minimise use of abbreviations, acronyms, contractions and symbols where possible; if used, they must be defined the first time in each entry eg Total Hip Replacement (THR)
- include an automatic digital signature using the staff member’s ‘HE number’ and name

If an entry has been written in error, staff will initiate a document tweak within the record as per the DMR User Guide.

Allied Health staff will not use another staff member’s log-in or identification to make an entry.

Allied Health Students will:

- Receive Read/Write Allied Health access to the DMR (as appropriate)
- Save their note as a Draft to enable supervisor review prior to submission

The Supervising Clinician will:

- Read through the draft prior to it being submitted and endorse or recommend changes
- Submit a separate progress note, under their HE number noting 'Progress note [date/time] by [Student Name] signed and approved'
- Complete the entry during the same shift as the students

Compliance, monitoring and evaluation

All staff are responsible for ensuring they document all patient interactions in the medical record. Compliance with this guideline will be monitored via annual documentation audits.

Related Policies / Documents

[WA Health Clinical Handover Policy](#)

[NMHS Clinical Documentation Policy](#)

[SCGH Clinical Handover Policy](#)

[Allied Health Guideline 11 Patient Identification](#)

[Allied Health Guideline 21 Abbreviations in Clinical Documentation](#)

Authorisation & Version Control

Policy Sponsor	Director Allied Health				
Policy Contact	Director Allied Health				
Date First Issued:	February 2017	Last Reviewed:	February 2025	Review Date:	February 2028
Version No.	7				
Approved by:	Director Allied Health			Date:	20/02/2025
Endorsed by:	Allied Health Executive Committee			Date:	20/02/2025
Standards Applicable	NSQHS Standards (Version 2): 				